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Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C St SW
Floor 7
Washington, DC 20201

Re: (RIN 0955-AA13) Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Assistant Secretary Keane,

The American Academy of Ophthalmology (the Academy) appreciates the opportunity to submit comments on the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

The Academy is the largest association of eye physicians and surgeons in the United States. A nationwide community of over 20,000 medical doctors, we protect sight and empower lives by setting the standards for ophthalmic education, supporting research, and advocating for our patients and the public. We innovate to advance our profession and to ensure the delivery of the highest-quality eye care.

We commend the U.S. Department of Health and Human Services (HHS) and ASTP/ONC for seeking stakeholder input on promoting the adoption of artificial intelligence (AI) in clinical care. Ophthalmology has long led AI innovation with tools already expanding screening access and enabling earlier detection of sight-threatening disease. The Academy is eager to provide feedback and looks forward to working with federal agencies to help scale these AI-driven tools responsibly and advance patient care and vision research.

Question 2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

There have been significant advances in the use of AI for improving screening, access and early detection of diabetic retinopathy (DR). DR is a common complication of type 1 and type 2 diabetes and is a preventable cause of blindness when detected and treated early.[1] DR is the leading cause of new cases of legal blindness among working-age Americans.[2] Data from 2019 indicates that 11.8 percent of adults with diagnosed diabetes reported severe vision difficulty or blindness.[3] [4]

Vision loss from DR is highly preventable with timely screening and access to treatment, as current therapeutic strategies are effective in preventing severe vision loss.[5] **Despite this, too many patients with diabetes still do not receive the recommended DR screening at the onset of their diabetes and during the subsequent years. In many cases patients have been unable unwilling to have an in-person recommended comprehensive eye exam.**

Recent advances in autonomous AI systems approved by the U.S. Food and Drug Administration have demonstrated the ability to meaningfully close this gap in quality care. Currently available imaging tools can screen for referable diabetic retinopathy with high sensitivity and specificity and nearly 100 percent negative predictive value. AI-based retinal screening tools can be deployed in primary care settings to greatly expand access to screening and identify patients who need timely referral to an ophthalmologist.[6] [7,8]

Expanding the use of validated AI-based retinal screening tools can substantially increase the rate of early detection of diabetic retinopathy for the population, enabling timely referral and preventing avoidable vision loss. **To realize the full benefit of these tools, HHS should prioritize efforts to increase awareness, adoption, and appropriate use of validated AI retinal screening tools to improve DR screening rates and reduce preventable vision loss among beneficiaries with diabetes.**

Ensure payment policy keeps pace with autonomous AI screening

One way that HHS can support broader adoption of these technologies is through payment policy. The Centers for Medicare and Medicaid Services (CMS) currently provides physician fee schedule pricing for CPT code 92229 (Imaging of retina for detection or monitoring of disease; point-of-care automated analysis and report, unilateral or bilateral), rather than utilizing carrier pricing. CMS has appropriately

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[1] Klein, B. E. (2007). Overview of epidemiologic studies of diabetic retinopathy. *Ophthalmic Epidemiology, 14*, 179–183.

[2] Kempen, J. H., O'Colmain, B. J., Leske, M. C., et al. (2004). The prevalence of diabetic retinopathy among adults in the United States. *Archives of Ophthalmology, 122*, 552–563.

[3] Lundeen, E. A., Saydah, S., Ehrlich, J. R., & Saaddine, J. (2022). Self-reported vision impairment and psychological distress in U.S. adults. *Ophthalmic Epidemiology, 29*, 171–181.

[4] Tonnies, T., Brinks, R., Isom, S., et al. (2023). Projections of type 1 and type 2 diabetes burden in the U.S. population aged <20 years through 2060: The SEARCH for Diabetes in Youth Study. *Diabetes Care, 46*, 313–320.

[5] Lim, J. I., Kim, S. J., Bailey, S. T., Kovach, J. L., Vemulakonda, G. A., Ying, G. S., Flaxel, C. J., & American Academy of Ophthalmology Preferred Practice Pattern Retina/Vitreous Committee. (2025). Diabetic retinopathy preferred practice pattern®. *Ophthalmology, 132*(4), P75–P162. https://doi.org/10.1016/j.ophtha.2024.12.020

[6] Abramoff, M. D., Lavin, P. T., Birch, M., et al. (2018). Pivotal trial of an autonomous AI-based diagnostic system for detection of diabetic retinopathy in primary care offices. *NPJ Digital Medicine, 1*, Article 39.

[7] Shah, A., Clarida, W., Amelon, R., et al. (2021). Validation of automated screening for referable diabetic retinopathy with an autonomous diagnostic artificial intelligence system in a Spanish population. *Journal of Diabetes Science and Technology, 15*, 655–663.

[8] Ipp, E., Liljenquist, D., Bode, B., et al. (2021). Pivotal evaluation of an artificial intelligence system for autonomous detection of referrable and vision-threatening diabetic retinopathy. *JAMA Network Open, 4*, e2134254.

recognized that practitioners incur significant ongoing resource costs associated with the software and infrastructure necessary to furnish this service.

We were encouraged that CMS requested stakeholder feedback on how modern medical technologies and services, such as Software as a Service (SaaS) and AI may be accurately valued in the Calendar Year 2026 Medicare Physician Fee Schedule Proposed Rule. As explained in our comment letter on that proposed rule, the image analysis fee is a direct practice cost analogous to any other supply because it is attributable to a specific imaging service provided to a specific patient each time the imaging is performed. It is not an indirect cost like computer hardware or software that is purchased once or licensed annually or monthly and then utilized repeatedly to provide varied services to multiple patients.

As the agency evaluates how to appropriately and accurately reimburse AI-enabled services, the Academy recommends that CMS consider and label such technology as a separate and new category within direct practice expense valuation.

Expanding diabetic retinopathy screening in FQHCs through AI-based tools

Federally Qualified Health Centers (FQHCs) are a vital resource for low-income and medically underserved communities, and their role in the health care system continues to grow. Between 2000 and 2018, the number of people receiving care at FQHCs increased by 196 percent, making FQHCs a main source of care for one in 12 Americans.[9] Yet few FQHCs currently offer screening for sight-threatening eye disease. Expanding diabetic retinopathy screening in FQHCs using validated AI-based technology presents a major opportunity to increase screening rates and reach patients who are least likely to receive timely eye care.

In 2024, the Academy launched an AI screening pilot program at Henrietta Johnson Medical Center in Delaware. In its first year, more than 30 percent of patients with a gradable image showed signs of diabetic eye disease requiring further ophthalmic evaluation. For many of these patients, screening at the FQHC was their first diabetic eye screening. These early results demonstrate how autonomous AI screening can identify disease earlier, trigger timely referral, and help prevent treatable vision loss. **The Academy plans to invest in additional programs and would welcome the opportunity to collaborate with HHS on expanding access to AI screening in FQHCs across the country building on our pilot projects.**

CMS also has a strong opportunity to improve access to sight-saving AI-based technology by supporting separate payment for CPT 92229 in FQHCs and Rural Health Centers. There is clear precedent for this policy. Glaucoma screening services, HCPCS codes G0117 and G0118, may be reimbursed as stand-alone billable visits for FQHCs and RHCs when no other services are furnished on the same day.[10] Extending similar payment policy to autonomous AI-based DR screening would improve access for underserved populations at high risk for vision loss.

Quality measures remain essential as screening gaps persist

Another way that HHS can promote the adoption of AI technologies is through the continuing use of quality measures. Substantial data demonstrate that diabetic retinopathy screening remains an unresolved gap in necessary medical care. According to the 2026 Medicare Part C and D Star Ratings Technical Notes, the Diabetes Care, Eye Exam measure had a numeric average of 77% and a Star Rating average of 3.4 out of 5 stars, showing that DR screening remains an active gap, not a solved problem. Similarly, data from the National Health Interview Survey found that only 66% of adults with diabetes had

[9] Sharac, J., Shin, P., Rosenbaum, S., & Handarov, T. (2019, September). *Community health centers continue steady growth, but challenges loom* (Policy Issue Brief No. 60). Geiger Gibson/RCHN Community Health Foundation Research Collaborative. Retrieved February 17, 2026, from https://geigergibson.publichealth.gwu.edu/60-community-health-centers-continue-steady-growth-challenges-loom

[10] Centers for Medicare & Medicaid Services. (2021, April 26). Medicare Benefit Policy Manual Chapter 13

a yearly eye examination in the past year, demonstrating little or no detectable change compared to prior years.[11]

CMS uses quality measures across its quality and payment programs to assess and incentivize high quality care, including measures that track diabetic retinopathy screening rates among patients with diabetes. Examples include Measure #117, Diabetes: Eye Exam, in the Merit-based Incentive Payment System program and the Diabetes Care, Eye Exam measure in the Medicare Advantage (MA) Star Ratings.

These measures provide essential ongoing education for physicians and patients on the importance of timely DR screening and establish incentives that protect Medicare beneficiaries from preventable vision loss. **Accordingly, the Academy strongly encourages HHS and CMS to retain these measures and continue using them to drive improvements in diabetes eye disease screening rates and early detection.**

Despite the ongoing need for these performance measures, CMS has recently proposed removing the Diabetes Care, Eye Exam measure from the MA Star Ratings as part of an effort to streamline the measure set. **The Academy strongly opposes this proposal, as eliminating this measure would remove a key incentive for MA plans and clinicians to prioritize diabetic retinopathy screening and referral.** This change risks delayed detection of disease, increased preventable vision loss, and higher long-term costs for patients and the Medicare program.

Question 8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Enhanced interoperability is essential to accelerating the development, validation, and deployment of AI in ophthalmology, particularly for imaging-based applications. Ophthalmology is uniquely positioned to benefit from interoperability improvements because the specialty relies heavily on high-resolution imaging modalities such as optical coherence tomography, fundus photography/imaging, and retinal angiography. These imaging data are foundational to both clinical decision-making and AI model development.

The Academy believes that Digital Imaging and Communications in Medicine (DICOM) is the foundational standard for ocular imaging interoperability. DICOM enables consistent capture of patient demographics, device metadata, acquisition parameters, and discrete quantitative measurements alongside image data. Broad adoption of DICOM is critical not only for safe and efficient clinical workflows, but also for enabling large-scale aggregation of high-quality, standardized imaging data.

Access to large, diverse, and well-annotated datasets is a requirement for developing robust AI models for clinical care. Interoperable imaging standards such as DICOM make it possible to aggregate data across health systems, vendors, and care settings, thereby reducing data fragmentation and enabling the scale required for AI training, external validation, and post-market monitoring.[12]

Despite these benefits, DICOM compliance remains low among ophthalmic imaging manufacturers and is a significant obstacle to achieving fully integrated digital workflows in ophthalmology.[13] Manufacturers remain hesitant to participate in the standards development process or to implement existing standards in their products due to concerns over the increased costs associated with DICOM compliance, the limited understanding among physicians regarding the benefits of DICOM, and the absence of regulatory

[11] U.S. Department of Health and Human Services, Office of Disease Prevention and Health Promotion. (n.d.). *Increase the proportion of adults with diabetes who have a yearly eye exam (D-04).* Healthy People 2030. Retrieved January 6, 2026, from https://odphp.health.gov/healthypeople/objectives-and-data/browse-objectives/diabetes/increase-proportion-adults-diabetes-who-have-yearly-eye-exam-d-04

[12] Gim, N., Ferguson, A. N., Blazes, M., Lee, C. S., & Lee, A. Y. (2025). The march to harmonized imaging standards for retinal imaging. *Progress in Retinal and Eye Research, 107*, 101363. https://doi.org/10.1016/j.preteyeres.2025.101363

[13] Goetz, K. E., Reed, A. A., Chiang, M. F., Keane, T., Tripathi, M., Ng, E., Nguyen, T., & Eydelman, M. (2024). Accelerating care: A roadmap to interoperable ophthalmic imaging standards in the United States. *Ophthalmology, 131*(1), 12–15. https://doi.org/10.1016/j.ophtha.2023.10.001

mandates requiring adherence to these standards. Additionally, the lack of third-party certification for DICOM conformance statements raises concerns about the extent of comprehensive compliance across the industry.[14]

To overcome these barriers, The Academy encourages ASTP/ONC and other federal agencies to take steps within their authority to accelerate imaging interoperability in ophthalmology, including exploring innovative approaches that promote compliance with DICOM standards. These standards are crucial to enabling digital workflows, supporting the sharing of high-quality images with patients, and advancing eye and vision research.

In response to these challenges, the National Eye Institute, Food and Drug Administration, and ASTP/ONC demonstrated their shared commitment to advancing DICOM standards by convening a workshop in 2022 aimed at incentivizing manufacturers to adopt these standards. [15] While this was a promising step, additional efforts are still needed to establish levers, benefits, and incentives that ensure stronger stakeholder and manufacturer support for the implementation of DICOM standards.

Conclusion

If you have questions or need any additional information regarding any portion of these comments, please contact Brandy Keys, MPH, Director of Health Policy at bkeys@aao.org or via phone at 202-587-5815. We look forward to ongoing engagement and stakeholder input.

Sincerely,

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American Academy of Ophthalmology

[14] Lee, A. Y., Sadda, S., Lum, F., et al. (2022). Joint American Academy of Ophthalmology and Association for Research in Vision and Ophthalmology policy statement: The time for digital imaging standards implementation is now. *Ophthalmology, 129*(11), 1229–1231. https://doi.org/10.1016/j.ophtha.2022.07.034

[15] Weixelbaum, J., Haqqani, S., Reed, A., & Goetz, K. (2022). *NEI-FDA-ONC joint workshop on promoting adoption of ocular imaging standards summary.* Zenodo. https://doi.org/10.5281/zenodo.6915785